Care Home Supply in Hawaii

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Challenge

The challenge is care home's low net profit with around the clock resident care makes it hard for care home owners to make a living solely by running the business. With projected elderly population of 85 years and older more than doubling next 20 years, where do they go when they need affordable care homes? From my personal experience, it was very hard to find a good care home for my late mother-in-law. It became a personal issue, and it is becoming a big issue among my friends and relatives.

Economic Concepts

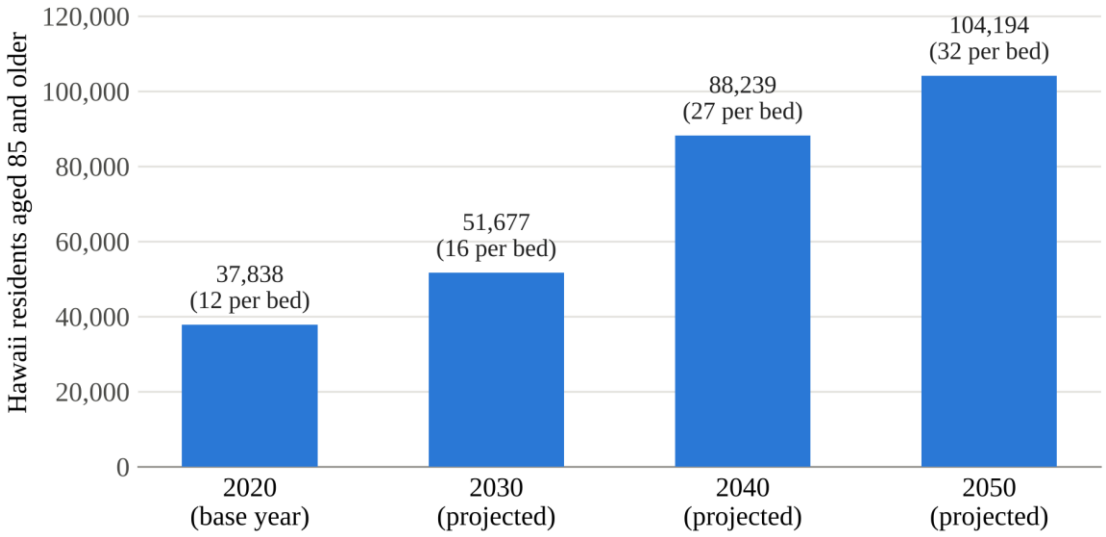
Net profit ~$35k-$60k for a 2-bed home vs. an RN's hospital pay of $136,320 ($65.54/hr)(State of Hawaii Department of Labor and Industrial Relations [DLIR], 2025) (opportunity cost; incentives) which discourages registered nurses from entering the field of care home business. I have registered nurse friends running a care home as a side business because the income it generates does not measure up to RN's hospital pay. It simply does not make economic sense. Care homes keep people out of costlier nursing homes (positive externality; public savings) however, if care home owners have to earn supplement income from another employment and healthcare benefits who's taking care of residents. GET charged on gross receipts, not profit (Tax burden on a low-margin business) According to care home analysis it can be from $3855 (10.9% of net profit of $35,411, personal estimation) to $5005 (8.4% of net profit of $59,828, personal estimation) which is significant portion of care home owner's low income.

Analysis

As Figure 1 shows, in the year 2030 in about a little more than three years from now projected number of Hawaii's 85 years and older population is 51,677 (Hawaii Department of Business, Economic Development and Tourism [DBEDT], 2024) which puts 16 people aged 85+ per bed in Hawaii's current care home beds count. And that number doubles in 20 years, 2050 to 104,194 (DBEDT, 2024) if certified beds stay at today's 3,281 (Hawaii Statewide GIS Program, 2026).

If nothing changes and beds numbers do not increase, more people end up in nursing homes at $101,000–$123,000 a year (Centers for Medicare & Medicaid Services [CMS], 2024) instead of about $45,100 (Hawaii Department of Human Services [DHS], 2025).

Figure 1. Hawaii's 85+ population vs. certified foster-home beds, 2020–2050



Note: "Per bed" divides the 85+ population by the 3,281 certified Community Care Foster Family Home beds (sum of capacity across 1,301 homes, April 2026), held constant. Sources: Hawaii DBEDT, State of Hawaii Data Book 2023, Table 1.24; Hawaii Statewide GIS Program, Community Care Foster Family Homes dataset (2026).

Recommendation

I am recommending the GET exemption because 10.9% of net profit for small business owners is big portion of added profit. The state sets the price with “inclusive of all applicable taxes” (DHS, 2025) and care home cannot pass GET on to anyone but pay themselves. It can be

argued that amount is not significant enough, but it is a great starting point into other measures like raising Level 2 rates. Act 47 added the exemption as HRS §237-24.3(12), effective January 1, 2026 (Hawaii Department of Taxation, 2025). However, the guidance lists eligible providers but doesn't define “health care facility” or name care homes (Hawaii Department of Taxation, 2025). Therefore, I am recommending the Legislature amends §237-24.3(12) to name community care foster family homes, ARCHs, and E-ARCHs explicitly. With that said, the objection will be the lost state tax revenue which is approximately $5,850,000 which is upper estimate due to not every home is full or all-Medicaid (1,300 homes X $4,500 which is my midpoint of $3,855–$5,005) from state database (Hawaii Statewide GIS Program, 2026). On the other hand, if Medicaid pays for a Level 2 resident going to a nursing home instead due to lack of availability of care homes, it will roughly cost $8500 to $10,200 (an approximate base rate built from the components) for a month about $101,000–$123,000 a year according to Hawaii's Medicaid State Plan Amendment HI-23-0014, effective January 1, 2024 (CMS, 2024). Compared to the care home for the same Level 2 is about $45,100 a year (DHS, 2025) which costs roughly $56,000–$78,000 less per year than in a nursing home. This lost state tax revenue of $5,850,000 divided by $56,000 lower of rough estimate of extra cost for higher cost nursing homes, comes out to 104. The state will break even on the tax revenue loss if 104 residents remain at the care homes. One thing to consider is though it is widely known that Medicaid costs are shared between federal and state governments so the state needs more than 104 residents to remain at care home to break even.

Conclusion

We need to have foresight in elderly care with it being very closely related to many local families with aging parents and grandparents. When there is clearly growing demand, we need to find a better solution for our working people supplying that demand with livable profit.

References

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